Afatinib monotherapy

Which treatment this guide covers NCCP 00221a covers adults with locally advanced or metastatic NSCLC with an activating EGFR mutation who have not received an EGFR TKI. The separate licensed squamous-cancer indication after platinum treatment is not included in this NCCP regimen or its evidence card.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Afatinib alone · daily oral treatment

Active medicines: afatinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.
- Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.
- Afatinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.
- Give pharmacy a list of all medicines, herbal products and supplements. Some P-gp inhibitors need separated dosing; strong inducers such as St John’s wort can reduce afatinib exposure. Ask for a written individual schedule before adding or changing medicines.
- Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 1 month after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____